

AHN Allegheny General Hospital

320 East North Ave, Pittsburgh, PA 15212

DISCHARGE SUMMARY Document #: DIS-015

Patient & Admission

Name	Evan Ashley
Date of Birth	1975-07-14
Phone	878-555-7527
Address	148 Eric Track Beaver, PA, 15009
MRN	MRN-543039
Admit Date	2026-04-17
Discharge Date	2026-04-21
Attending	Bruce McMahon, MD
Attending NPI	2427868015

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	HSB784 801845

Primary Discharge Diagnosis

Coronary artery disease s/p elective LHC

ICD-10: I25.110

Hospital Course

The patient presented for elective left heart catheterization following an abnormal stress test and recurrent exertional chest discomfort. Baseline echocardiography demonstrated preserved left ventricular ejection fraction at 55% with no significant wall motion abnormalities. Coronary angiography revealed significant stenosis of the left anterior descending artery with moderate disease in the left circumflex, consistent with multivessel coronary artery disease. The right coronary artery was patent. Given the angiographic findings and the patient's symptom burden, percutaneous coronary intervention to the LAD was performed with successful placement of a drug-eluting stent. The procedure was uncomplicated with TIMI III flow restored and no residual stenosis noted. Troponin levels remained negative post-procedure, and the patient tolerated the intervention well without hemodynamic compromise.

During the hospital stay, the patient remained hemodynamically stable on dual antiplatelet therapy with aspirin and ticagrelor. Serial electrocardiograms showed no acute ischemic changes. She was initiated on guideline-directed medical therapy including a high-intensity statin, beta-blocker, and ACE inhibitor for secondary prevention and cardioprotection. The patient's post-procedure course was uncomplicated with no access site complications or arrhythmias. She ambulated without limitation and reported significant improvement in her baseline chest discomfort. Discharge planning included comprehensive patient education

regarding medication adherence, particularly the importance of dual antiplatelet therapy for a minimum of twelve months, and lifestyle modifications including cardiac rehabilitation referral.

The patient is being discharged in stable condition with close outpatient cardiology follow-up scheduled for two weeks post-discharge. She has been counseled on warning signs requiring urgent evaluation and provided with written discharge instructions and medication reconciliation.

Procedures Performed

Coronary angiography with FFR	CPT 93458
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Medication Changes

Atorvastatin 40 mg daily	ADJUSTED
Clopidogrel 75 mg daily	STARTED
Isosorbide mononitrate 30 mg daily	STARTED
Aspirin 81 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	10 days
Urgency Tier	ROUTINE
Clinical Flags	radial access — wrist precautions, femoral access — bleed precautions

Dictated and electronically signed by:
Bruce McMahon, MD
NPI: 2427868015
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